

Patient Medical Record

BrainConnect AI - Sample Test Document

July 7, 2026

Patient Information

Full Name	Sarah Johnson
Patient ID	BC-2026-001
Age	52 Years
Gender	Female
Height	165 cm
Weight	78 kg
Blood Group	O+
Occupation	Office Administrator
Date of Visit	July 7, 2026

Chief Complaint

Patient reports persistent chest discomfort, fatigue, and shortness of breath during mild physical activity over the past two weeks.

History of Present Illness

The patient describes intermittent pressure-like pain in the center of the chest lasting approximately 10 to 15 minutes. Symptoms are occasionally associated with sweating and mild dizziness. Pain improves with rest.

No recent trauma reported.

Past Medical History

- Type 2 Diabetes Mellitus (8 years)
- Hypertension (10 years)
- Hyperlipidemia
- No previous cardiac surgery

Family History

- Father died from myocardial infarction at age 61.
- Mother has hypertension.
- Older brother diagnosed with coronary artery disease.

Current Medications

- Metformin 1000 mg twice daily
- Lisinopril 20 mg daily
- Atorvastatin 20 mg daily
- Aspirin 81 mg daily

Vital Signs

Blood Pressure	148/92 mmHg
Heart Rate	96 bpm
Respiratory Rate	18 breaths/min
Temperature	36.8°C
Oxygen Saturation	96%
BMI	28.7

Laboratory Results

Fasting Blood Glucose	176 mg/dL
HbA1c	8.2%
Total Cholesterol	245 mg/dL
LDL Cholesterol	162 mg/dL
HDL Cholesterol	39 mg/dL
Triglycerides	215 mg/dL
Troponin I	Slightly Elevated
Creatinine	0.9 mg/dL

Radiology Report

Chest X-Ray:

No focal lung consolidation.
Cardiac silhouette is mildly enlarged.
No pleural effusion.

Echocardiogram:

- Mild left ventricular hypertrophy.
- Left ventricular ejection fraction approximately 50%.
- No significant valvular abnormalities.

ECG Findings

- Normal sinus rhythm.
- Mild ST-segment depression in leads V4–V6.
- No acute ST elevation.

Allergies

No known drug allergies.

Lifestyle

- Former smoker (quit 5 years ago)
- Occasional alcohol consumption
- Sedentary lifestyle
- Diet high in processed foods

Physician Notes

The patient presents with multiple cardiovascular risk factors including diabetes, hypertension, hyperlipidemia, obesity, and a strong family history of coronary artery disease. Clinical presentation is concerning for stable ischemic heart disease. Further evaluation with coronary CT angiography or cardiac catheterization is recommended.

Medical management should focus on optimization of blood pressure, glycemic control, lipid reduction, and lifestyle modification.